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2024062570844

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Patient's Name [redacted] PIMS No 2024062570844

Admission Notes

Date & Time	
10/3/26	c/s/B [redacted]
12:00PM	A 6½ year old male child came with c/o Fever (3/3/26) x 7 days. c/o vomiting x 6 days non projectile non bilious. c/o loose stools x 5 days. c/o cough & Rhinorrhoea x 7 days c/o poor oral intake. c/o decrease urine output. No c/o burning micturation. c/o Irritability. No c/o altered sensorium. No c/o fast breathing
	Past history :- Nil Significant.
	O/E ⇒ child was active & alert
	HR – 114/min C/E ⇒ : eschar (R) Iliac region.
	RR – 22/min
	SpO2 – 98%.
	R/S ⇒ B/L air entry NVBS.
	CVS ⇒ S1S2
	CNS ⇒ NFND
	P/A ⇒ Soft non-tender Liver span 11cm.
	Δ Fever under evaluation Plan
	: Enteric fever ⇒ To send CBC, CRP, blood culture, widal, urine
	: Scrub. LFT, RFT
	⇒ To send H1N1 swab Scrub typhus

[redacted]

[redacted]

Master [redacted]

PROGRESS SHEET

2024062570844

Patient's Name..... PIMS No .....

Date & Time	
	Plan
	⇒ IV Ceftriaxone 100mg/kg/day.
	⇒ IV Pantoprazole OD
	⇒ IV Emeset SOS.
10/03/26 6.30 pm	S/B [redacted] (AR)
	chief complaint,
	fever spikes ⊕. 104.4 F @ 1.30 pm.
	loose stools x 2 ope.
	no vomiting since admission
(D1) Inj. Ceftriaxone 100mg/kg/day.	Abd: soft, non-tender.
	liver is palpable.
	spleen tip palpable
	Plan.
	- Continue IV antibiotics.
	- to f/c blood c/s.
	- w/F fever spikes & vomiting episodes
	- to add oral Doxycycline.

[redacted]

[redacted]

2024062570844

Patient's Name ..... PIMS No .....

Date & Time	
11/3/26	C/S/B [redacted]
DOT-8	Δ Tropical Fever. Probable Enteric fever ? Scrub typhus.
	⇒ c/o fever x 3 episodes high grade.
	⇒ c/o ↓ oral intake
	⇒ No c/o lethargy
Based on history & examination ⇓ [No features of complication]	⇒ No c/o Irritability
	⇒ c/o Vomiting x1 episode.
	⇒ c/o loose stools x 3 episodes
[U/O ⇒ 1.2ml/kg/day]	⇒ No c/o altered Sensorium
	⇒ No c/o bone pain
	⇒ No c/o ↓ U/O
	⇒ No c/o abnormal movements.
	⇒ No c/o abdominal pain
O/E ⇒	HR - 132/min [Temp - 102.4°F]
	RR - 34/min
	SpO2 - 99% CP/PP +++/++
S/E ⇒ CVS ⇒ S1S2	P/A ⇒ soft non tender
CNS ⇒ NFND	Liver is palpable 1cm. Spleen 1cm RCM

Spleen tip is palpable

[redacted]

Master [redacted] ESS SHEET

Patient's Name .....2024062570844 .....PIMS No .....

Date & Time	
	Plan ⇒ To continue IV ceftriaxone ⇒ To continue antipyretics ⇒ To continue oral doxycycline ⇒ To continue IV fluids. ⇒ To trace blood reports widal serum, dengue reports.
14/03/26 10:00AM	C/S/B - [redacted]
	Child Reviewed, C/o fever C/o Vomiting C/o Loose stools No C/o lethargy No C/o irritability No C/o Altered Sensorium

[redacted]

[redacted]

PROGRESS SHEET

Patient's Name .....PIMS No .....

Date & Time	
	O/E: Child ~~x~~ Alert, Active PR: 128/min RR: 30/min SpO2: 98% @ Room Air
NO eschar NO lymphadenopathy No rash	SIE: -- P/A: - Soft, non tender, spleen tip palpable. liver span 11cm. RS: BIL AE (+) CNS: S1 S2 (+) CNS: No focal neurological deficit
	U/O - 1.3ml/kg/hr
	Plan: 1) To Send Urine Culture 2) To collect widal & Scrub Report today. Follow up Blood c/s Taper IV fluids Encourage oral.

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PROGRESS SHEET

Patient's Name .....PIMS No .....

Date & Time	
11/2/26 6 pm	c/s/B [redacted]
	- Child reviewed
	- No fresh issues
	o/e - Alert
Last 6 hrs	- Last fever spike 6:30 pm 103.3F
2.5 ml/kg/hr	- Afebrile x 12 hrs.
Loose stools	- s/E - NAD
4 episodes	- Plan: - Follow Blood c/s reports
	- Encourage ~~Pt~~ Oral intake
	- Continue s/o IV fluids
	- ORS after each loose stool.
[redacted]	- HR, BP, RR, pulse volume, SpO2, urine o/p, CRPP s/o dehydration } 04 hrly

[redacted]

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PROGRESS SHEET

Patient's Name ..... PIMS No .....

Date & Time					
11/3/26			Monitoring		
5 pm					
	PR	RR	BP	SPO_2	CP/PP
5 pm	128/min	30/min	100/60 mm Hg	98%	++/++
9 pm	110/min	25/min	110/80 mmHg	99%	++
1 Am	115/min	27/min	110/80 mm Hg	99 %	++
5 Am	100/min	28/min	110/70 mmHg	98%	++
7 Am	108/min	26/min	110/70 mmHg	98%	++

[redacted]

[redacted]

Master, [redacted]

SS SHEET

2024062570844

Patient's Name ..... PIMS No .....

Date & Time	
12/03/26 7:00AM	C/S/B - [redacted]

Δ = ENTERIC FEVER

⇒ c/o fever at 1:30AM (104°F)
⇒ No c/o abdominal pain
⇒ No c/o ↓ oral intake
⇒ No c/o lethargy
⇒ No c/o poor oral intake.
⇒ No c/o Irritability
⇒ No c/o Vomiting.
⇒ No c/o lethargy
⇒ No c/o headache & abnormal movements
⇒ No c/o reduced urine output.

O/E ⇒ child was active & alert
HR - 96/min
RR - 28/min
SpO2 - 97%.
BP - 100/60 mmHg.

S/E :- CVS ⇒ S1S2
CNS ⇒ NFND
P/A ⇒ Soft nontender Liver is palpable 2cm below RCM 10.5 cm. Liver span
R/S ⇒ B/L air entry NVBS

[redacted]

Master [redacted] SHEET

Patient's Name ..... 2024062570844 ..... PIMS No .....

Date & Time	
	Plan ⇒ To collect widal reports ⇒ To collect blood culture. ⇒ To continue IV antibiotics.
12/8/26 9:30 am	CLSIB- [redacted] Δ = ENTERIC FEVER — No complications C/o Fever at 1:30pm (101°F) No other complaints O/E:- Child; Alert, Active PR: 102/min RR: 22/min SpO2: 98% @ Room Air S/E:- P/A: Soft, non tender, ~~No organomegaly~~ Hepatosplenomegaly (+) RS: B/L AE (+) CNS: Tone (N) CVS: S1S2 (+) Plan: 1) To collect widal & Blood Culture Reports 2) Watch for danger signs 3) To collect Urine Culture Reports

[redacted]